

August 20, 2026

BlueCross BlueShield of South Carolina
Provider Appeals Department
P.O. Box 100300
Columbia, SC 29202

RE: Formal Appeal of Claim Denial — Willis Crona (MRN MUSC8226170), Claim MUSC-7099-3

Patient	Willis Crona (DOB 1946-10-19)
MRN	MUSC8226170
Member ID / Group	BCB494161219 / GRP-79505
Claim number	MUSC-7099-3
Date of service	2026-04-18
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$464.99
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-08-29

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this formal first-level reconsideration on behalf of Willis Crona (Member ID: BCB494161219, Group: GRP-79505) regarding the denial of Claim MUSC-7099-3 for services rendered on April 18, 2026, by Priya N. Raghavan, MD (NPI 1669504132) at MUSC Health Ashley River Tower. BlueCross BlueShield of South Carolina denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, citing a Local Coverage Determination, and further stated that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." The total denied amount is \$464.99 for one unit of CPT 99214, an established outpatient office visit of moderate complexity. MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the allowed amount of \$246.13.

CLINICAL BACKGROUND

Mr. Crona is a 79-year-old male with a complex, active medical history that directly informs the clinical context of the April 18, 2026 encounter. His active conditions include carcinoma in situ of the prostate and prostate neoplasm, for which he is currently receiving leuprolide acetate and docetaxel; atrial fibrillation managed with warfarin sodium, verapamil hydrochloride, and digoxin following multiple electrical cardioversions and catheter ablations between 2019 and 2021; microalbuminuria due to type 2 diabetes mellitus; and chronic kidney disease stage 2. The presenting diagnosis for the April 11 visit was J06.9, acute upper respiratory infection, unspecified, with a secondary diagnosis of E66.9. In a patient of this age and complexity — particularly one anticoagulated with warfarin, receiving active oncologic therapy, and carrying known renal impairment — evaluation of even a seemingly routine acute illness requires careful clinical judgment to assess for drug interactions, infection severity, and systemic complications.

BASIS FOR APPEAL

The denial rests on the assertion that the documentation does not establish medical necessity for the service billed. MUSC Health contends that this conclusion is inconsistent with the clinical facts and with generally accepted standards for evaluation and management coding. CPT 99214 reflects an established-patient outpatient visit of moderate complexity, a level of service that is appropriate when the presenting problem requires consideration of multiple comorbidities, review of an active medication regimen, and clinical decision-making that carries meaningful risk. Mr. Crona's concurrent use of warfarin, digoxin, verapamil, leuprolide, and docetaxel creates a pharmacological environment in which an acute upper respiratory infection is not a trivial matter; the risk of drug interactions, secondary bacterial infection, and hemodynamic instability in the setting of atrial fibrillation and renal impairment elevates the complexity of any acute visit well beyond a simple, low-complexity encounter. The medical record supports that Dr. Raghavan, as the rendering provider, exercised the level of clinical judgment commensurate with a moderate-complexity visit. The payer has not identified any specific documentation deficiency, nor has it indicated what additional clinical elements would satisfy its standard. MUSC Health is prepared to submit the complete encounter documentation, including the physician's clinical notes, medication reconciliation, and problem list, in support of this appeal.

REQUESTED ACTION

MUSC Health respectfully requests that BlueCross BlueShield of South Carolina conduct a thorough review of the complete medical record for the April 18, 2026 encounter, reverse the medical necessity denial issued on May 31, 2026, and reprocess Claim MUSC-7099-3 for payment at the contracted allowed amount of \$246.13. Should the payer require additional clinical documentation or a peer-to-peer review with Dr. Raghavan, MUSC Health welcomes that opportunity and will respond promptly. This reconsideration is submitted in advance of the appeal deadline of August 29, 2026, through the My Insurance Manager portal as directed on the remittance advice.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record